

Crisis Stabilization Plan and Follow-Up Records

Participant: Mara Ellison **Service span:** February 4–10, 2025 **Related facility:** Alder Creek Regional Medical Center outpatient crisis service **Civil exhibit lodged:** December 12, 2025

This fictional compilation documents voluntary crisis planning after Ellison’s emergency evaluation. It is included to show the welfare context, functional aftermath, and follow-up chronology. It neither labels Ellison as dangerous nor reconstructs the disputed hand movements during police contact.

On February 4, Ellison met with an outpatient crisis clinician for medication reconciliation, symptom review, and a short safety plan. She described feeling overwhelmed on February 3 after missing scheduled medication and experiencing racing thoughts. She said she wanted distance from Iris and became more distressed when police arrived. She denied that she had possessed a weapon, threatened anyone, planned self-harm, or intended to flee.

Ellison reported soreness at her left wrist and shoulder and tenderness at two dressed contact sites. She said emergency imaging showed no fracture. The clinician encouraged her to follow the medical discharge instructions and to separate physical-care questions from crisis counseling. Ellison identified sleep, reduced stimulation, medication consultation, and one trusted contact as immediate needs.

February 4 stabilization plan

Ellison and the clinician created a written plan using concrete, voluntary steps. Early warning signs included missed sleep, difficulty sorting competing demands, withdrawing from ordinary communication, and feeling crowded by multiple speakers. Helpful responses included one person speaking at a time, a direct choice between two manageable options, a quieter setting, and time to answer without repeated commands.

Ellison agreed to contact her prescribing clinic that day to review the missed dose rather than changing medication independently. She chose Iris as the family contact who could check in by text before arriving. Iris agreed not to touch or restrain Ellison during distress and to call the crisis line for guidance unless an immediate emergency required emergency services.

The plan listed crisis-line and urgent-care contacts, transportation options, and signs requiring emergency evaluation. Those signs included a stated intent to harm self or another, inability to maintain basic safety, serious medical symptoms, or an observed weapon. The clinician documented that none was present during the February 4 appointment.

Ellison also planned short walks, hydration, regular meals, and temporary reduction of work demands. The plan did not promise that every future encounter could be managed without police. It provided information that responders could use when circumstances permitted and emphasized that mental-health symptoms alone do not establish violent behavior.

February 6 combined medical and crisis follow-up

At the February 6 contact, Ellison reported improved sleep and completion of medication reconciliation. She denied current self-harm or harm-to-others thoughts. She remained upset by the police encounter and described startle responses when hearing radio sounds. The clinician recorded those statements as symptoms and did not diagnose a permanent trauma condition at this early visit.

Ellison's wrist abrasion was healing without drainage. The two superficial contact areas remained tender but showed no spreading redness or blister complication. Shoulder movement had improved, although overhead reaching and carrying a bag remained uncomfortable. She continued conservative measures and had no new numbness, weakness, or swelling. No fracture had been found at emergency imaging.

Ellison and Iris reviewed communication boundaries. Iris explained that her call was intended to obtain calm welfare assistance, not arrest or punishment. Ellison agreed that Iris could ask whether she wanted a crisis clinician, transportation, or quiet presence before calling emergency services when no immediate danger existed. They did not attempt to decide whether Rusk's account of the physical encounter was accurate.

The plan remained voluntary. Ellison scheduled another check on February 10 and received instructions to seek earlier care if physical or mental-health symptoms worsened. No involuntary hold, criminal charge, or weapon concern was recorded.

February 10 follow-up and transition

Ellison attended the February 10 follow-up and reported taking medication as directed after consultation with her prescriber. Thought organization, sleep, and appetite had improved. She denied an immediate safety concern and identified work, family, and legal appointments she could manage with ordinary outpatient support. The crisis service transitioned her back to established care rather than extending acute monitoring.

The left-wrist abrasion was nearly closed, and the contact sites were dry and improving. Ellison continued to describe shoulder soreness with some movements but reported greater range and less pain than at the emergency visit. She had not developed weakness or sensory loss. She was reminded to seek medical reassessment if recovery stalled; the crisis clinician made no independent orthopedic diagnosis.

Ellison discussed the difference between recalling the event and interpreting the camera. She consistently said she pulled away because the wrist grip hurt and denied striking Rusk or reaching for his belt. The clinician noted that this was Ellison's account. No media was reviewed, and the session did not test perception or credibility.

The closing plan retained Iris as an optional contact, the crisis-line number, and Ellison's preference for one speaker and short questions during distress. Ellison received a copy. The record does not suggest that needing a crisis plan made her dangerous on February 3 or that later improvement proves what happened during the obscured interval.

Custodian certification and scope

The fictional outpatient-record custodian certifies that these entries accurately represent the authored February 4, 6, and 10 contacts. They were produced as a single civil exhibit on December 12, 2025. Routine scheduling and billing material is omitted. The remaining text preserves the clinical chronology, physical-symptom follow-up, participant statements, and voluntary plan.

The service's purpose was stabilization, not forensic investigation. Clinicians did not interview Rusk or Vega, inspect the breezeway, download a device, or analyze recordings. Ellison's consistent denial of a strike or reach is relevant as her recorded account, but treatment repetition does not transform it into independent eyewitness proof. Likewise, her mental-health symptoms provide context without deciding force reasonableness.

The physical facts remain aligned with the emergency record: wrist abrasion, two superficial contact burns, shoulder strain, improving symptoms, and no fracture. This compilation does not introduce a new injury or claim permanent impairment. It also records no weapon, criminal charge, or immediate threat during follow-up.

Alder Creek Regional Medical Center and its crisis service are fictional. The certification represents only an invented court exhibit. It is not an actual health record, treatment recommendation, diagnosis for any real person, or legal opinion about the seizure, excessive force, immunity, or appellate review.